June 16, 2025

The Honorable Mehmet Oz, MD, MBA
Administrator
Centers for Medicare and Medicaid Services
Baltimore, MD

Thomas Keane, MD
Assistant Secretary for Technology Policy
U.S. Department of Health and Human Services
Washington, DC

Dear Administrator Oz and Assistant Secretary Keane:

On behalf of the Healthcare Information and Management Systems Society (HIMSS), we are pleased to provide public comments on the Centers for Medicare and Medicaid Services (CMS) and Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology (ASTP) Health Technology Ecosystem Request for Information (RFI).

HIMSS is a global advisor, thought leader and member-based society committed to reforming the global health ecosystem through the power of information and technology. As a mission-driven non-profit, HIMSS offers an unique depth and breadth of expertise in health innovation, public policy, workforce development, research, and analytics to advise global leaders, developers, end users, patients, and influencers on best practices in health information and technology driven by health equity. Through our innovation engine, HIMSS delivers key insights, education and engaging events to healthcare providers, governments, and market suppliers, ensuring they have the right information at the point of decision. HIMSS serves the global health information and technology communities with focused operations across North America, Europe, the United Kingdom, the Middle East, and Asia Pacific. Our members include more than 127,000 individuals, 480 provider organizations, 470 non-profit partners, and 650 health services organizations. Our global headquarters is in Rotterdam, The Netherlands and our Americas headquarters is in Chicago, Illinois.

To accelerate digital health transformation, we must foster seamless, secure, ubiquitous, and systemwide data access and interoperable health information exchange. Interoperable information exchange enables the right people to have the right access to the right health information in an usable format at the right time to deliver optimal care. Health information and technology serve as the catalyst for transforming the health ecosystem, modernizing care delivery, driving health

innovation, improving efficiency, and enabling health research. Actionable information that helps identify the best pathway for each patient is critical to help Make America Healthy Again.

The following are HIMSS's responses to the questions posed in this RFI:

What are the most promising areas for digital health innovation that could benefit Medicare beneficiaries and how can CMS and other agencies foster the development and adoption of effective and equitable digital health solutions?

In a recent HIMSS study, hospitals with advanced digital maturity demonstrated stronger quality and safety outcomes. These hospitals can all mobilize data from multiple external sources, automate the tracking of patient progress and health outcomes, and engage patients directly with their health information. Hospitals with advanced digital maturity were also more likely to have more advanced safety structures and systems, and stronger quality and safety outcomes in support of workforce capacity challenges. This wide array of digital health interventions, alongside accessible and accurate information can significantly impact patient care outcomes and quality of life. Many care settings (small provider offices, clinics, etc.) are still trying to achieve a higher degree of digital sophistication.

Technologies that have shown promise in helping all care settings include:

Artificial Intelligence

Artificial intelligence (AI) drives numerous applications that offer the potential to improve patient outcomes, accelerate early detection of disease, and enhance point of care, administrative and operational efficiency. AI tools are positioned to drive innovation and digital transformation.

For effective deployment and use of AI to improve the health and human services sector, the broader digital health infrastructure must be supported in sustainable ways. AI and related technologies rely heavily on a robust and mature digital infrastructure that can produce quality information and inform health and care decisions.

Artificial intelligence is utilized in various ways across healthcare organizations. According to the 2024 HIMSS Healthcare Cybersecurity Survey, 37% of respondents (representing healthcare cybersecurity professionals) reported using AI for technical tasks like support and data analytics, 35% for clinical services such as diagnostics, and cybersecurity and administrative tasks (each 34%). AI is also used in healthcare operations (31%), research (24%), patient engagement (21%), and clinical training (16%). Additional use cases are anticipated in the future as AI rapidly advances. HIMSS 2024 AI in Healthcare survey also shows that AI progress is greatest in larger, better-resourced organizations, with half of larger healthcare organizations reporting AI

projects in the early implementation stages versus only one-quarter of smaller organizations. These data correlate with digital maturity, with larger organizations demonstrating higher values.

Policies and regulatory frameworks should find a balance that promotes and accelerates the responsible deployment and use of safe and trusted AI demonstrated to benefit patients, providers, and other participants in the health ecosystem, while ensuring that AI is continually monitored and revalidated following deployment in the field. Specifically, the Department of Health and Human Services (HHS) should consider the HIMSS Global Artificial Intelligence Policy Principles and HIMSS's response to the White House Office of Science and Technology Policy Artificial Intelligence Action Plan RFI when developing a regulatory framework for the deployment and use of AI. At minimum, HHS should adopt policy frameworks that:

- Leverage policy guardrails based on specific use case and risk level;
- Focus both on the development cycle and the revalidation of AI models once they have been deployed and have evolved after being exposed to local data; and
- Ensure AI and other automated tools should, at a minimum, not amplify harmful biases, and at best, help address any biases already inherent in health data

In addition, HIMSS encourages HHS to monitor the growing divide between large, well-resourced health systems and small organizations, particularly those organizations with small margins that care for at-risk and underserved communities and take steps to ensure that the benefits of AI are available to healthcare organizations across the entire spectrum of care.

Telehealth

Telehealth is a safe, effective and essential method of expanding access to care delivery — particularly to specialists — for countless at-risk patients, including those in rural and underserved communities. Telehealth can drive operational efficiency, reduce the burden of the care team, shape patient engagement, and improve quality and safety. Telehealth, especially when used for mental health services, plays a key role in managing public health, maintaining continuity of care, and ensures the sustainability of health care.

The alignment of reimbursement and incentives to expand the deployment and use of telehealth as a tool to improve care access is critical to digitally transforming healthcare. The temporary regulatory changes that enabled telehealth services to be properly reimbursed by Medicare should be extended until Congress can permanently ensure clinicians can provide millions of Americans access to high-quality virtual care.

Finally, HIMSS encourages HHS to work with Congress to clarify reimbursement for telehealth services. The lack of certainty that telehealth services will be reimbursed at the same rate as in-person services is putting a strain on providers.

Data Visualization

Providers now have more data and information to drive healthcare decision-making than at any point in history. The exponential growth of available information has created a challenging environment for providers as they struggle to identify the key performance indicators that should have the most impact on patient care decisions. Currently, much of the information that is generated by health systems is not truly mobilized to inform decisions. Analytics in the healthcare sector are relatively underdeveloped compared to other business sectors. Actionable insights from information lead to impactful results. HIMSS's 2024 AI Survey shows analytics as one of the three primary use cases organizations are considering or have implemented AI solutions. Therefore, public policy should incentivize the use of visualization tools to drive better care outcomes and performance.

HIMSS developed the Analytics Maturity Assessment Model (AMAM) to support and guide organizations to realize the full potential of analytics to strengthen their data-driven decision-making at the patient and enterprise levels. AMAM begins with foundational requirements of data, digital infrastructure and governance to support these health organizations advance their digital health transformation and enable successful adoption of advanced analytics and AI to help achieve organizational priorities and outcomes.

By encouraging organizations to benchmark their analytics and other measures with such tools, we all can gain a stronger understanding of the roadmap each organization will need to follow to achieve transformation.

Digital Quality Measurement

Since 2012, HIMSS has actively requested that CMS support the proliferation and use of electronic clinical quality measures (eCQMs). The core of HIMSS's mission is promoting the use of information and technology to improve the quality of healthcare delivery through effective performance measurement and clinical decision support. HIMSS's recommendations have reinforced that eCQMs provide a more meaningful and actionable measure of care quality than traditional claims-derived quality measures. Corresponding recommendations have focused on ensuring eCQMs are appropriately tested to ensure accuracy in all care settings, drive improvements in patient outcomes, and are not overly burdensome to collect within a normal clinical workflow.

With CMS focused on transitioning all reporting programs to digital quality measures (dQMs), HIMSS looks forward to continuing to engage with CMS to ensure measures are

feasible, accurate, actionable, and not overly burdensome to collect. HIMSS supports CMS' goals of transitioning all CMS quality reporting programs to leverage digital health technology for less burdensome and more accurate results. These goals should occur with less latency in reporting through secondary use of clinical data captured and/or mapped to common national FHIR data standards via a common standards-based dQM framework) As HIMSS represents the largest community of digital health and quality measures implementers, HIMSS is launching a diverse Digital Measure Adoption Work Group with representation from developers, providers, measure developers, and payers to create guidance to support the widespread adoption of dQMs and remove adoption barriers.

CMS must address significant barriers before a complete transition to FHIR API-enabled dQM-reporting can take place. These barriers include, but are not limited to:

- A lack of sufficient eCQM's/dQM's availability for many clinical specialties and a corresponding high cost of between $1 million and $2 million per measure for development, testing, and endorsement, and a slow development timeline ranging from two to seven years
- The lack of a compelling business case for diverse end user participation in real world testing of the measures before including them in a CMS program
- CMS currently requires ASTP health IT certification for all data sources used in eCQM calculations. While this is valuable for clinical data quality, it should not apply to the subset of financial/billing data used to supplement clinical data in accurate dQM calculations. Those data should be acceptable if they comply with the UB-04 standards for hospital billing and CMS-1500 for provider billing standards.
- The developer and implementer communities need clarification regarding a singular national standard for dQM's. An unambiguous resolution on whether HL7 FHIR Qi-Core or USCDI+ Quality will be used as the national dQM implementation standard. A clear rationale and process to harmonize the two are necessary in case both standards are required to support certification and technical implementation guide use cases.
- The fundamental issue of assuring data quality that feeds accurate dQM calculation engines and its reporting still has not been fully addressed.

HIMSS recommends that CMS take the following actions:

1) Collaborate with ASTP to launch a pathway to merge the clinically nuanced data elements available in USCDI+ quality domain with future versioning of USCDI. Accordingly, CMS should announce the timeline for sunsetting legacy reporting formats, including QRDA, and align these timelines with adoption milestones for FHIR-based dQM across all programs. This will ensure smoother transitions and reduce the burden associated with reporting using multiple methodologies. This will require some level of government funding or

sustainability training for providers and health systems. If the government is not prepared to provide funding, HHS should partner with industry to educate provider and health system constituencies on non-government sustainability resources.

2) While the development and feasibility testing of future versions of USCDI is taking place, a small number of dQMs are currently available that use underlying data elements that are found in the latest versions of USCDI. HIMSS recommends that those measures be made available as voluntary reporting measures, with scoring bonuses to eligible hospitals and critical access hospitals who choose to report those measures. Examples include (add language when available.)
3) Provide funding support for pilot projects to ensure that the testing cohort for new and/or substantively updated eCQMs and dQMs include a significant sample size, including large and small hospitals across a wide geographic and patient demographic spectrum.
4) Provide a significant scoring bonus for hospitals participating in testing in the Inpatient Quality Reporting program and newly developed value-based care models. This heightens critical access hospitals, rural hospitals, and community hospitals opportunities to receive incentive program dollars.
5) Implement a requirement for CMS-funded future measure development and testing contracts to allocate sufficient funding to facilitate testing, mapping, and implementation work for field testing at testing sites. As mentioned in the first recommendation, if the government is not prepared to fund the improvements, then HHS should work with providers and health systems to train them on how to find sustainable resources.

What safeguards are needed to ensure the responsible and ethical use of digital health technologies in Medicare?

Advancing cybersecurity, information security, physical security, and information privacy measures in healthcare organizations serving Medicare beneficiaries should be supported to: 1) protect the confidentiality, integrity, and availability of patient information and other sensitive information and assets of participants in the health ecosystem, 2) ensure the continued and effective delivery of patient care and coordination of care, 3) protect patient safety and privacy, and 4) further the delivery of safe, secure, and effective care delivery and coordination.

HIMSS suggests leveraging the 2024 HIMSS Healthcare Cybersecurity Survey Report for insights into the necessary cybersecurity safeguards. In addition, earlier this year, HIMSS responded to Office for Civil Rights HIPAA Security Rule proposed rule. HIMSS emphasized the importance of different approaches to make security, risk assessment and documentation requirements more scalable to the security needs of small practices and practices caring for rural and underserved communities and their business associates.

Building upon HIMSS's framework for assessing Digital Determinants of Health, CMS should require any beneficiary-facing application or AI-enabled service to demonstrate basic accessibility and clear usability. Conformance with Section 508 of the Rehabilitation Act, plain-language materials validated at or below a sixth-grade reading level, and culturally appropriate instructions will help ensure that no eligible beneficiary is left behind because of disability or low digital literacy. Embedding these requirements in coding or reimbursement criteria will advance consistent quality of care and reduce avoidable program waste.

Consistent with the HIMSS AI/ML Public Policy Principles, specifically Principle 7 on *Human Oversight and Transparency*, Principle 3 on Clinician and Consumer Trust, and Principle 4 on Monitoring and Evaluation, CMS should require that any high-stakes digital health functionality (e.g., diagnostic decision support, therapy modification, or automated prior authorization) include a clearly documented human-in-the-loop checkpoint and an auditable manual-override pathway. This safeguard preserves clinician accountability and patient autonomy while permitting automation to streamline administrative workflows and improve throughput. Moreover, by mandating human oversight and systematic performance monitoring, CMS would operationalize HIMSS's call for explainable tools that maintain fair, robust performance across beneficiary populations and provide a feedback loop for real-world safety signals.

CMS should move toward a policy that ties reimbursement to open, standards-based APIs (FHIR R5, SMART on FHIR), adherence to information-blocking prohibitions, and transparent data-portability metrics. Such requirements support competition, facilitate beneficiary choice, and prevent costly data silos.

What are the current barriers to data interoperability that hinder the effective use of digital health products and the sharing of health information among stakeholders?

While cost, workflow complexity, data quality, standard-variance, misaligned clinical priorities, and even data-blocking behaviors are all real-world impediments, these are symptoms of a deeper economic misalignment. Interoperability remains under-funded precisely because private health systems and electronic health record (EHR) vendors are not compensated for delivering what is a public good. These barriers include:

- **Economic Misalignment** Under current payment models, hospitals and health systems absorb the full cost of implementing, maintaining, and operating data exchange infrastructure with little corresponding revenue or subsidy. As Medicare and Medicaid reimbursement rates drift farther below true cost of care, the ability to invest in exchange platforms, data-mapping efforts, and care coordination workflows is increasingly constrained, and CMS' programs for incenting interoperability lose their effectiveness. Without direct subsidies or a sustainable business model for data exchange, true plug-and-play interoperability remains commercially unattractive.

- **Technical Barriers Are Nearing a Margin-Zero Problem** Thanks to limitless cloud compute, mature API-first platforms, and rapidly advancing AI-based transformation tools, the "last mile" of unifying disparate documentation formats, mapping legacy clinical codes, and normalizing structured and unstructured data can now be largely automated at near-zero marginal cost. In the long run, there is the potential that generative AI and natural-language-understanding engines will render format translation and terminology reconciliation effectively invisible, eliminating the need for data dictionaries or expensive custom interfaces. Until such time, standards-based information access should be promulgated by HHS.

Regarding the effective use of digital health tools, CMS should recognize that effective use of digital tools among older Americans often hinges on live support. A 2021 study shows that live, community-based coaching meaningfully improves older adults' digital engagement and cuts avoidable utilization. In HIMSS Member University of California, San Francisco's Video Visits for Elders Project, 18.7 percent of patients ≥ 65 requested technical coaching and, once assisted, the share completing their appointment by video more than doubled to 40 percent of those reached. A 2023 meta-analysis of seven trials found face-to-face digital literacy training significantly improved eHealth literacy among older adults. Complementing these engagement gains, a randomized controlled trial in a Massachusetts safety-net system showed that 30-day patient-navigator support reduced readmissions by four percentage points in patients over 60, demonstrating tangible downstream savings from navigator programs. HIMSS urges CMS and ASTP to pilot coverage for community-based "digital health navigators," for example, through expanded chronic-care management codes. Hands-on assistance will help seniors adopt technology confidently, improving engagement and reducing downstream costs from misuse or non-use.

What steps can be taken to improve data exchange and integration across different health information systems?

HIMSS supports integration between all interoperability approaches, entities, and trusted exchange frameworks, including current standards-based approaches, cloud scale and AI-driven data harmonization when demonstrated to be effective, to reduce technical barriers to interoperability. HIMSS urges CMS and ASTP to rebalance incentives, through targeted subsidies, procurement policy, and AI-friendly certification so that information exchange becomes both technically and economically self-sustaining.

HIMSS recommends ASTP and CMS act on the following:

1. **Introduce Targeted Exchange Subsidies:** Consider a time-limited add-on to existing quality or care-coordination payments that explicitly cover the costs of data sharing, thereby removing the risk for vendor and provider investment.
2. **Encourage Vendor Neutrality Through Procurement Rules:** Incorporate data-portability requirements and AI-readiness criteria into Certified EHR reimbursement policy, so that future certification cycles reward open, cloud-native, AI-compatible designs, at least for standard API and interface implementations.
3. **Leverage AI to Lower Technical Friction:** Fund phased programs that demonstrate how generative AI pipelines can automatically harmonize clinical content across settings and use those pilots to validate financial models that tie reimbursement directly to interoperable data exchange.
4. **Monitor and Adjust Over Time:** Establish clear outcomes metrics for data-exchange uptake and ROI and commit to sunset dates on subsidies or add-ons as the market matures and AI-enabled automation reduces per-transaction costs.
5. **Test and Certify**: Programs should be incorporated into policy frameworks to increase market confidence in the interoperability of health information. These programs should be developed with extensive input from relevant subject matter experts and impacted parties.

Information Exchange with Public Health

As part of the push to democratize data, public health needs to be included as a stakeholder. Notably, the RFI requests feedback from multiple perspectives, but not from public health. CMS and ASTP should continue to coordinate with the Centers for Disease Control and Prevention (CDC) and state, territorial and local agencies to ensure public health data systems are modernized and public health and health care organizations are exchanging critical information, to more efficiently improve community health and wellbeing, prevent disease, and respond to emergencies. In addition, HIMSS encourages ongoing investments in HIE and interoperability infrastructure, exemplified by ASTP's State HIE Cooperative Agreement and Medicaid HITECH 90/10 Federal Financial Participation.

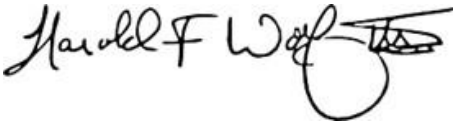
Many states are developing and remodeling their data governance and exchange services to support broader community needs. Utilizing existing technical and relationship infrastructures across states to bring together personally identifiable health data, as well as population and public health data may support community-based health interventions for chronic disease, and improve well-being based on the application of robust analytics. HIMSS's members have consistently shared that there is a need for this; however, challenges remain, including low incentives and the need for

more resources. HIMSS leads a Public Health Data Modernization Advisory and Community for members to share best practices and recommendations in this regard.

Interoperability with public health entities poses unique challenges that require novel solutions. HIMSS recommends no longer prescribing one-size-fits-all standards for each of the 4,000+ state, local, tribal and territorial public-health agencies and 3,000+ hospitals to replace, modify, or reconfigure their systems. Instead, HIMSS urges HHS to build on the success of the Pandemic Ready Interoperability Modernization Effort, by deploying a lightweight, cloud-native normalization layer, like the AI-enhanced FHIR translator the current DOGE Administrator, Amy Gleason, and the U.S. Digital Service team prototyped for COVID reporting. With this kind of infrastructure, HHS can meet every jurisdiction where they are, ingesting CSVs, HL7 v2 feeds, PDFs and more, and dynamically harmonizing them into a single canonical format. This approach avoids the prohibitively expensive and infeasible mandate of having thousands of disparate organizations adopt identical APIs and instead creates a resilient, demand-driven bridge to CDC/NIH analytics delivering low-marginal-cost interoperability without waiting years for universal standard adoption.

HIMSS looks forward to the opportunity to discuss these issues in depth with you and your teams. Please feel free to contact Jonathan French, Senior Director of Public Policy and Content Development, at Jonathan.French@HIMSS.org with questions or to request more information.

Sincerely,



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